

Community Water and Sanitation Access in Sub-Saharan Africa

An Analysis of WHO/UNICEF Joint Monitoring Programmes (JMP) 2025 Data

Prepared by: Clement Etim

Date: May 2025

Data Source: WHO/UNICEF Joint Monitoring Programme (JMP) Global WASH Dataset 2025

Executive Summary

Access to clean water, basic sanitation, and hygiene facilities remains one of the most pressing development challenges in Sub-Saharan Africa. This report presents findings from an analysis of the WHO/UNICEF Joint Monitoring Programme (JMP) 2025 dataset, covering 45 countries across the region from 2000 to 2024.

The findings show that while meaningful progress has been made over the past two decades, large gaps remain. On average, 7 in 10 people have access to basic water, fewer than half have basic sanitation, and fewer than 1 in 3 have access to a basic handwashing facility. Progress on hygiene has been particularly slow, and the gap between rural and urban communities continues to hold back overall development across the region.

Background and Context

Water, sanitation, and hygiene, collectively known as WASH, are fundamental to human health, dignity, and economic development. Without access to clean water and proper sanitation, communities face higher risks of waterborne diseases, child mortality, school dropout, and lost productivity.

Sub-Saharan Africa carries a disproportionate share of the global WASH burden. Despite decades of investment from governments, international organisations, and donors, millions of people across the region still lack access to services that most of the world takes for granted.

This analysis draws on data from the WHO/UNICEF JMP, which is the globally recognised standard for tracking WASH progress. The dataset covers 45 Sub-Saharan African countries and spans 25 years of data from 2000 to 2024, making it one of the most comprehensive sources available for understanding trends and disparities across the region.

Key Findings

1. Water Access Has Improved, But Safely Managed Water Remains Out of Reach

Basic water access across Sub-Saharan Africa has improved significantly over the past two decades, rising by an average of 17.5 percentage points since 2000. Today, 70.2% of the regional population has access to at least a basic water source.

However, the story is quite different when it comes to access to safely managed water, which is defined as water that is clean, accessible on premises, and reliably available. Only

28.9% of the population has safely managed water, and data for this indicator is missing for nearly half of the countries in the region, reflecting how difficult it remains for many governments to monitor and report on water quality.

The best performers on basic water access are Cabo Verde (92.3%), Botswana (91.8%), and South Africa (90.2%). At the other end, the Democratic Republic of the Congo (35.7%), the Central African Republic (36.5%), and South Sudan (39.7%) remain severely underserved.

2. Sanitation progress is uneven, and open defecation continues to be a public health concern.

Basic sanitation access has improved by an average of 17.6 percentage points since 2000, but at 40.0%, it remains well below the level needed to protect public health. More than half the population of Sub-Saharan Africa still lacks access to even a basic toilet facility.

Open defecation, the practice of defecating in the open rather than using a toilet or latrine, affects an average of 17.6% of the regional population, with some countries still reporting rates above 60%. Eritrea (67.0%), Niger (66.0%), and South Sudan (62.8%) have the highest rates in the region.

Several countries have made significant strides in reducing open defecation. Ethiopia reduced its open defecation rate by 55.6 percentage points over the period, driven by a well-documented national sanitation programme. Cabo Verde and Somalia also recorded major reductions.

Conversely, the Central African Republic and the Democratic Republic of the Congo have seen open defecation rates increase since 2000, which is as a result of the devastating impact of prolonged conflict on basic services.

3. Hygiene Access Is the Most Neglected WASH Pillar

Of the three WASH pillars, hygiene has received the least investment and shown the least progress. Only 27.6% of the regional population has access to a basic handwashing facility, and the average improvement since the mid-2000s stands at just 6.5 percentage points, far below the gains recorded for water and sanitation.

The worst performers on hygiene access include Liberia (3.5%), Ethiopia (5.9%), and Burundi (6.4%). These figures are deeply concerning given what we know about the role of handwashing in preventing the spread of infectious diseases.

Kenya stands out as the strongest performer on hygiene among large mainland countries, with 57.5% basic hygiene access, an improvement of 42.3 percentage points since 2005.

4. The Rural-Urban Gap Is One of the Most Persistent Challenges

Across all three WASH indicators, people living in rural areas are consistently worse off than those in urban areas. The average gap in basic water access between urban and rural communities stands at 28 percentage points across the region.

Eritrea has the largest water access gap at 61.9 percentage points, followed by Equatorial Guinea (47.3) and Burkina Faso (47.1). For sanitation, Mauritania has the largest gap at 54.8 percentage points.

A small number of countries have managed to close this gap significantly. Sao Tome and Principe, Comoros, and Cabo Verde all have rural-urban water gaps below 10 percentage points. Rwanda is a particularly notable case; rural Rwandans actually have better sanitation access than urban Rwandans, a result of deliberate government policy that prioritises rural communities.

It is worth noting that a small rural-urban gap does not always reflect success. In countries like Liberia and Sierra Leone, the gap is small primarily because urban areas are also severely underserved. True progress requires both closing the gap and raising overall access levels.

5. Conflict and Fragility Are the Strongest Predictors of Poor WASH Outcomes

In every indicator analysed, conflict-affected and fragile states consistently appear at the bottom of the rankings. South Sudan, the Central African Republic, Chad, the Democratic Republic of the Congo, and Eritrea record some of the lowest access rates in the region for water, sanitation, and hygiene alike.

These countries also show the least progress over time and, in some cases, have recorded declines in access since 2000. This underscores the reality that no amount of investment in WASH infrastructure can outpace the destruction caused by sustained conflict.

Sub-regional Comparison

Eastern and Southern Africa perform slightly better than West and Central Africa on most WASH indicators. The East records higher averages for basic sanitation (42.8% vs 38.1%) and safely managed water (32.3% vs 26.4%). West and Central Africa have a marginal edge on basic water access and hygiene.

Both sub-regions face significant challenges, and neither is on track to meet the Sustainable Development Goal 6 target of universal access to safe water and sanitation by 2030.

Recommendations

Based on the findings of this analysis, the following actions are recommended for NGOs, donors, and governments working in the WASH sector across Sub-Saharan Africa:

1. **Prioritise hygiene funding.** Hygiene has been consistently underfunded relative to water and sanitation. Given its critical role in disease prevention, increased investment in handwashing infrastructure and behaviour change programmes is urgently needed.
2. **Close the rural-urban gap.** Infrastructure investment must be directed specifically at rural and hard-to-reach communities. Programmes that serve only urban populations will widen existing inequalities rather than reduce them.
3. **Strengthen data collection for safely managed water.** Nearly half of Sub-Saharan African countries lack data on safely managed water access. Without this data, it is

impossible to understand the true quality of water services or track progress meaningfully.

4. **Increase support for conflict-affected states.** Countries in fragile and conflict-affected situations require dedicated WASH programming that accounts for the instability of their operating environments. Standard delivery models are not sufficient in these contexts.
5. **Learn from high performers.** Countries like Rwanda, Kenya, and Ethiopia have demonstrated that low income does not have to mean poor WASH outcomes. Their approaches to community-led sanitation, rural prioritisation, and government accountability provide lessons that others can adapt across the region.

Limitations

This analysis is based on modelled estimates produced by the WHO/UNICEF Joint Monitoring Programme (JMP). While these estimates represent the best available global data, they carry uncertainty, particularly for countries with limited survey coverage. Data for safely managed water and sanitation is missing for a significant number of countries, and hygiene data is only available from 2005 onwards. When interpreting the findings, it is important to consider these gaps.

Conclusion

Sub-Saharan Africa has made real progress on WASH access over the past two decades. Millions of people have gained access to water and sanitation services that their parents did not have. But the pace of progress is not fast enough, and the distribution of that progress has been deeply unequal.

Closing the gap between rural and urban communities, addressing the neglect of hygiene, and finding ways to deliver services in conflict-affected settings are the defining WASH challenges of the next decade. The data is clear about where the problems are. What is needed now is the political will and the funding to act on it.

This report was produced as part of a data analytics portfolio project. All data are sourced from the WHO/UNICEF Joint Monitoring Programme (JMP) Global WASH Dataset 2025. Full methodology, clean datasets, and an interactive dashboard are available on request.

